QUICK REFERENCE · FAMILY MEDICINE GUIDES

Counselling in Family Medicine

Five evidence-based strategies for the 5–10-minute consultation

Choose the strategy

- **Ready and receptive** → Five A's or FRAMES
- **Motivation is the problem** → identify the stage, then motivational interviewing
- **Emotional / psychosocial** → BATHE ± medication
- **No response after 2–3 brief interventions** → refer
- Several behaviours can be addressed together; use the **teachable moment** (acute illness, pregnancy, new diagnosis).

Stages of change

Stage	Recognise	Your goal
Precontemplation	No plan within 6 months	Raise awareness; respect choice
Contemplation	Considering within 6 months; ambivalent	Explore pros and cons
Preparation	Acting within 30 days	Concrete plan and date
Action	Changed, <6 months	Reinforce; problem-solve
Maintenance	Changed, >6 months	Prevent relapse; normalise lapses

Stage-matched interventions are no better than non-staged ones (Cochrane) — use the stage to choose the conversation, and re-ask every visit.

The Five A's

Ask	Every visit, every product
Advise	Clear, personal, linked to today — "I recommend…"
Assess	Knowledge and willingness to try now
Assist	Plan, date, medication, support, leaflet
Arrange	Follow-up booked — the step most often missed

Not ready? The 5 R's: Relevance · Risks · Rewards · Roadblocks · Repetition. **30 seconds?** Ask · Advise · Act.

FRAMES

Feedback	Personal risk with their own numbers
Responsibility	"The choice is yours."
Advice	Clear, neutral, anchored to a standard
Menu	Several options to choose from
Empathy	Warm, reflective, non-judgemental
Self-efficacy	Build on past success

Ask permission before sharing information. Good for harm reduction and chronic-disease feedback.

Refer — brief counselling is not enough

- Suicidal thoughts, plan or intent
- Psychosis, mania, severe depression
- Dangerous withdrawal risk (alcohol, benzodiazepines, opioids)
- Domestic violence or child-protection concern
- Eating disorder with physical instability
- No response after 2–3 brief interventions

The effective counsellor

Empathy · genuineness · unconditional positive regard · active listening · self-awareness · respect for autonomy · confidentiality · cultural humility · hope

Motivational interviewing

- **Spirit:** partnership · acceptance · compassion · empowerment (evocation)
- **Four tasks:** engaging → focusing → evoking → planning
- **OARS:** Open questions · Affirmations · Reflections · Summaries
- **Change talk:** DARN (Desire, Ability, Reasons, Need) → CAT (Commitment, Activation, Taking steps). Respond with EARS.
- **Elicit–Provide–Elicit** to give information; **two reflections per question**; "and", not "but".
- Resist the **fixing reflex**; reflect sustain talk, never argue.

Readiness ruler (0–10)

- Importance 4 or below → work on importance (values, discrepancy).
- Importance 5+ but confidence 4 or below → build self-efficacy.
- Both 7 or above → plan.
- Ask **"Why 6 and not 3?"** — never "why not 10?"

BATHE — about 5 minutes

Background	"What's been going on in your life?"
Affect	"How do you feel about that?"
Troubles	"What troubles you most about it?"
Handling	"How are you managing?"
Empathy	"That sounds really hard."

Not a full history. Don't assume the trouble. Problem-focused coping for the solvable; emotion-focused for the uncontrollable.

Tobacco cessation in 10 minutes

- Ask about **cigarettes, shisha, vapes, medwakh, smokeless**.
- Clozapine / olanzapine levels rise on quitting — reduce and monitor.
- Quit date within 2 weeks · follow up within 1 week · book the **Sehhaty** cessation clinic.

Heaviness of Smoking Index

Points	First cigarette after waking	Cigarettes / day
3	within 5 min	31 or more
2	6–30 min	21–30
1	31–60 min	11–20
0	after 60 min	10 or fewer

Total 0–1 low · 2–4 moderate · 5–6 high. Cigarettes only.

Medicine	Regimen	Caution
Varenicline	Start 1–2 wk before quit date: 0.5 mg daily days 1–3 · 0.5 mg twice daily days 4–7 · then 1 mg twice daily, 12 weeks	Nausea, dreams; CrCl <30 reduce
Combination NRT	21 mg/24 h patch if >10 a day or first cigarette within 30 min (else 14 mg) + gum, lozenge or inhaler; 8–12 weeks	Skin irritation; safe in stable CVD
Bupropion SR	Start 1–2 wk before: 150 mg daily × 3 days, then twice daily, 7–12 weeks	Not with seizures, eating disorder, MAOI
Cytisine	WHO 2024 recommended	Check local availability

Evidence in numbers

- MI in primary care: **SORT A** (weight, BP, alcohol); 10–15% added benefit. For smoking, Cochrane 2019: low-certainty, no clear benefit alone.
- Physician advice: quitting 8.0% vs 4.8%. Advice + medication + support: 15.2% vs 8.6% (USPSTF 2021, A).
- Five A's, FRAMES, BATHE, stages: **SORT B**.
- Saudi adults using tobacco: 19.8% (GATS 2019) — men 30.0%, women 4.2%.

Searight, Am Fam Physician 2018 · USPSTF 2021 · WHO tobacco cessation guideline 2024 · Cochrane 2010, 2014, 2019 · Miller & Rollnick 2023 · Stuart & Lieberman · GATS 2019 · MOH